

Case Study 1

Name	Mr B
DOB	1970
Occupation	Self employed builder prior to incident
Claim form	October 2010 Fell 6 metres from a balcony while on a stag do in Spain. Head injury. Amnesia, loss of the last year's memory.
Medical Evidence	No recollection of events 12-24 hours prior and approx 1 week after the event When regained consciousness was confused and more aggressive October 2010 CT Brain in Spain – normal. November 2010 – UK Neurologist referral. Mr B felt he had made a good recovery and thought he was 'just about back to normal' but had been lacking in confidence since the injury. Nervous about returning to work. Considered he had no personality problems and said he should be allowed to drive. Wife did report changes in personality (argumentative/aggressive) and short term memory – nervous about him returning to driving (causing conflict). Consultant suspected he, at least, had concussion and probably a post concussion syndrome where individuals have changes in memory and personality. No treatment. Told physically capable of driving, but not to until behaviour normalised. Diagnosed with 'post concussion syndrome'. October 2011 – UK Psychologist – Since the accident has had x2 family bereavement and suffering depression from bereavement and impacts of the accident/memory loss. Had made some improvement in cognitive function when not in times of stress, but stress has impaired recovery. Should improve further but thought to be only able to recover about 50% of pre-accident cognitive function. Has been able to undertake some simple work, but unable to take any complex work and unlikely to be able to in the future. October 2012 – Passed driving test, found work as a parcel delivery driver. Unable to do work as a builder, had been working in a manufacturing job but unable to continue. MMSE 29/30 with a deficit in recall memory February 2013 – Neuropsychologist report. Clear pattern of inconsistent effort on some tests but findings consistent with cognitive difficulties following a diffuse axonal injury. Likely to be compounded by emotional problems experienced. Symptoms of depression could have jeopardised the performance of the tests. March 2013 – Treating Consultant Neurologist – Sx likely to be 70% as a result of the accident and 30% indirect psychological problems. March 2014 – Second Consultant Neurologist - The injuries suffered and short term memory problems are consistent with brain injury. Sx of mild cognitive deficit in the form of reduced memory still present 40 months after injury and therefore likely to be permanent. Requires prompts to manage day to day life. Manages work well as allows for regular prompts. Emotional problems following the injury are no longer a significant issue.

CI Definition	Traumatic brain injury – <i>resulting in permanent symptoms</i> Death of brain tissue due to traumatic injury resulting in permanent neurological deficit with persisting clinical symptoms.
Questions	Is there evidence of death of brain tissue? Is the CI definition fulfilled? If yes, when do you have enough evidence to support the decision